NCCP 00320 · VERSION 6

Your treatment, explained

Progressive unresectable or metastatic pancreatic NET, or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c. · NCCP 00320 v6 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Everolimus · neuroendocrine tumours

Daily treatment, with four-weekly safety blood tests

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for new or worsening cough, breathlessness or fever, mouth ulcers limiting eating or drinking, marked thirst, much less urine, severe diarrhoea or signs of infection. Severe breathing difficulty or collapse needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · smpc

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
How your treatment fits together
Your 28-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Your everolimus treatment
Daily tablets, checks and mouth care
When to contact your team
Understanding your particular NET
Making sense of tests and results
Hormone symptoms and day-to-day changes
Your overall plan and choices
Eating, energy, practical help and support
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

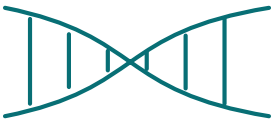
How your medicines work

Everolimus is a daily targeted tablet used to help control particular advanced NETs. It aims to slow growth; the team balances disease control, side effects and your priorities. This NET guide does not cover the separate kidney-cancer indication.

TARGETED THERAPY TABLET

Everolimus

Afinitor



Reduces mTOR growth signalling

Everolimus blocks a growth-control pathway called mTOR inside cells. This can slow NET growth. Healthy tissues and immune responses also depend on these pathways, helping explain mouth ulcers, infections, lung inflammation and changes in blood sugar or fats.

mTOR signal → Growth pathway blocked → Tumour growth may slow

What to watch for

Report mouth ulcers and new breathing symptoms early; keep blood and metabolic checks even if you feel well.

Sources: smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Mouth / eating

Everolimus

Mouth ulcers may appear early. Use prescribed mouth care; report soreness, pain or difficulty eating and drinking.

3 Blood / infection

Everolimus

Blood counts can fall. Fever, shivering, unusual bruising or sudden illness needs urgent advice even without a scheduled visit.

5 Kidneys / fluid

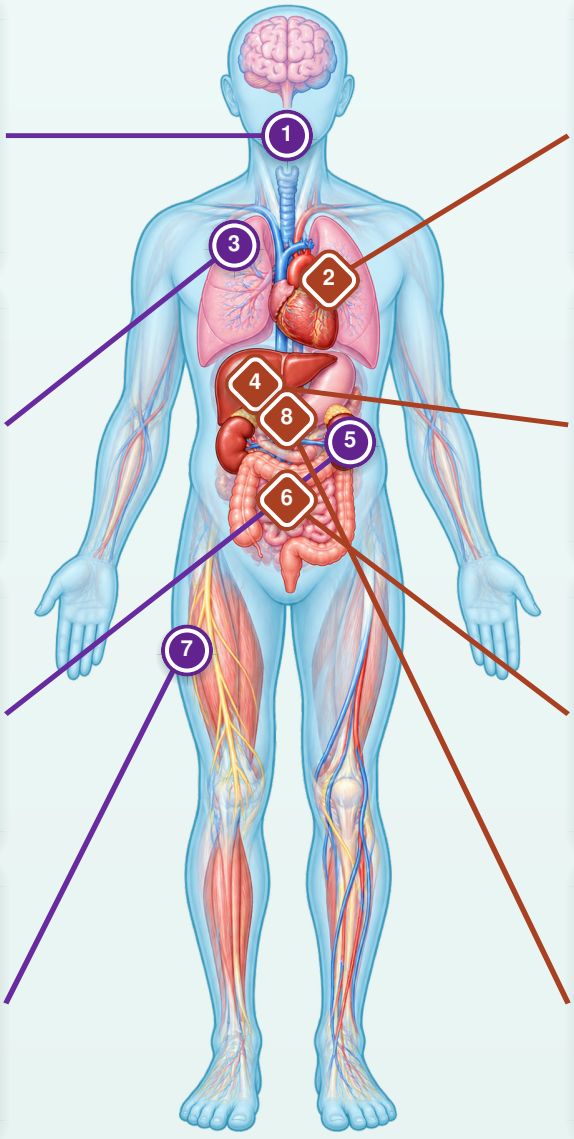
Everolimus

Kidney problems or protein in urine may need extra tests. Report swelling, dehydration or much less urine.

7 Skin / wounds

Everolimus

Rash and slow wound healing can occur. Report severe rash or unhealed wounds and discuss surgery before it is arranged.



2 Lungs / breathing

Everolimus

New cough, fever or breathlessness may be infection or lung inflammation. Contact oncology promptly; severe breathlessness is an emergency.

4 Liver

Everolimus

Tests can change before symptoms occur. Attend blood checks and report yellow skin or eyes, dark urine or persistent abdominal pain.

6 Bowel / appetite

Everolimus

Diarrhoea, nausea or poor appetite can affect fluids and weight. Report persistent symptoms or inability to drink.

8 Blood sugar / energy

Everolimus

Blood sugar and fats may rise without symptoms. Report marked thirst, frequent urine or unusual fatigue; keep the agreed tests.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

How your treatment fits together

CONTINUOUS

Daily everolimus

Daily | 4-week checks

Continue daily only while the team confirms benefit and tolerability. Blood reviews do not create a planned tablet break.

The written prescription and team review determine treatment; this timeline is a reference.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, NET response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00320 v6 · current Irish regimen · EMA product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Everolimus · neuroendocrine tumours: treatment and checks

Progressive unresectable or metastatic pancreatic NET, or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c.

Take the prescribed everolimus dose once daily every day, at the same time and consistently with or without food. There is no planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00320	FBC, renal/liver tests, BUN and urine protein	FBC, kidneys and liver every 4 weeks	Earlier assessment if unwell, dehydrated or less urine	Review abnormalities before continuing	Follow unresolved effects

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Glucose / blood fats NCCP 00320	Fasting glucose, cholesterol and triglycerides	Lipids regularly; fasting glucose as clinically indicated	Report thirst, frequent urine or marked fatigue	Review treatment for diabetes or lipid changes	Individual follow-up
Infection / mouth / lungs NCCP 00320	Review active infection; hepatitis B surface antigen and core antibody	Ask about ulcers, cough, breathlessness, infection and rash	New breathing or infection symptoms need prompt contact	Assess lung inflammation versus infection	Follow symptoms after a hold or stopping
NET response / hormone plan NCCP 00320	Document tumour site, grade, symptoms and baseline assessment	Scans / markers only as individually scheduled	Track symptoms and separate hormone medicines	Review benefit, burden and personal goals	Agree next treatment and follow-up

What each check is for

1

Daily everolimus

Continuous

Everolimus is a daily targeted tablet used to help control particular advanced NETs. It aims to slow growth; the team balances disease control, side effects and your priorities. This NET guide does not cover the separate kidney-cancer indication.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00320

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take the prescribed everolimus dose once daily every day, at the same time and consistently with or without food. There is no planned tablet-free week.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Everolimus every day	Days 1–28	Continue into the next review window unless your team tells you to hold.
Mouth, breathing and infection watch	Days 1–28	Report changes when they happen.
Four-week safety review	Day 28	Blood count, kidneys and liver; other checks on your individual plan.

There is no planned rest week: The 28-day visual is a review window. Do not take a routine week off or continue through a team-directed hold.

Lung effects and infection can look similar: A new cough or breathlessness needs assessment; do not assume it is a harmless tablet effect or start leftover antibiotics or steroids yourself.

Monitoring continues after changes: Ask who follows unresolved symptoms or abnormal tests after a delay, dose change or stopping.

> **Why these bands stay broad:** Treatment timing is shown for orientation. Symptoms can develop on any day; no symptom band predicts an individual toxicity peak. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00320 v6 · current Irish regimen ↗

> **There are no guaranteed "safe days".** Follow your oncology unit's emergency instructions. Fever, shivering or suddenly feeling unwell needs urgent advice; do not wait for the next appointment. Call emergency services for collapse, severe breathing difficulty, chest pain or uncontrolled bleeding.

Complete timing and symptom notes

Day 1–7: Starting safely

Take the prescribed daily dose consistently with or without food. Start the mouth-care plan and check other medicines. Confirm the first blood-test date and who to call.

Day 8–28: Continue daily; report symptoms early

Mouth, breathing, infection, bowel or skin symptoms can occur at any time. Do not wait for the four-week review. Attend blood checks and follow any written hold or dose-change instructions.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking tablets

The usual starting dose is 10 mg once daily, but follow your own prescription. Swallow whole with a glass of water; do not chew or crush. Take consistently with or without food. If a dose is missed, do not take an extra dose: take the next prescribed dose at the usual time.

STEP 2

Medicines, vaccines and food

Avoid grapefruit and St John's wort. Ask the pharmacist to check all prescribed, over-the-counter and herbal products. Live vaccines should be avoided during treatment; agree vaccination with your team.

STEP 3

Procedures and radiotherapy

Discuss planned surgery, wounds and current or recent radiotherapy. Everolimus may affect healing and can worsen or reactivate radiation-related inflammation. The teams agree any interruption.

STEP 4

Safe storage and supply

Keep tablets in their original labelled packaging, out of children's reach, and follow the pack storage instructions. Return unused tablets to the pharmacy as advised. Plan repeat prescriptions before your supply runs out.

STEP 5

Pregnancy, feeding and fertility

Tell the team before starting if pregnant, planning pregnancy or breastfeeding. Ask for the medicine-specific contraception duration and fertility advice, and record it in your plan.

STEP 6

Review visits

Baseline blood count, kidneys/liver, BUN, urine protein, lipids, fasting glucose and hepatitis B screening. Blood count, kidney and liver tests every four weeks. Lipids regularly and glucose as clinically indicated; scans and NET markers follow the consultant plan.

Sources: NCCP 00320 v6 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Mouth care

Use gentle mouth care and any prescribed alcohol-free mouthwash. Ask before buying a rinse; avoid products containing alcohol, peroxide, iodine or thyme. Report ulcers early.

2 Sickness and bowel support

Everolimus has minimal-to-low emetogenic potential. Routine premedication is not required by this regimen; agree any rescue anti-sickness or diarrhoea medicine with the team.

3 Infection prevention

Existing infection must be assessed and resolved before starting. Hepatitis B testing informs any specialist prevention plan. Vaccination and any antimicrobial prevention, including PJP prevention with certain steroid plans, are individual decisions.

4 Eating and energy

Report weight loss, poor intake or persistent fatigue. Ask for nutritional and symptom support; do not continue through severe symptoms simply to finish a pack.

Sources: nccp · smpc

TREATMENT REFERENCE · 01

Your everolimus treatment

Daily treatment with regular safety reviews.

Why everolimus?

Everolimus is a daily targeted tablet used to help control particular advanced NETs. It aims to slow growth; the team balances disease control, side effects and your priorities. This NET guide does not cover the separate kidney-cancer indication.

Sources: nccp · smpc

The exact Irish NET indication

Progressive unresectable or metastatic pancreatic NET, or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c. The GI indication is nonfunctional; do not transfer it to lung NET or renal cell carcinoma.

Sources: nccp · smpc

How it works

Everolimus blocks a growth-control pathway called mTOR inside cells. This can slow NET growth. Healthy tissues and immune responses also depend on these pathways, helping explain mouth ulcers, infections, lung inflammation and changes in blood sugar or fats.

Sources: nccp · smpc

Trial evidence and limits

Everolimus delayed progression compared with placebo in these trial populations. That does not mean cure or predict an individual response. The studies do not provide one personal survival estimate. RADIANT-3 final overall survival did not show a statistically significant difference, with crossover complicating interpretation. RADIANT-4 combined GI and lung populations; its inclusion of lung NET does not broaden NCCP 00320c.

Sources: radiant3 · radiant3-os · radiant4

TREATMENT REFERENCE · 02

Daily tablets, checks and mouth care

Keep the current prescription with your treatment record.

Taking tablets

The usual starting dose is 10 mg once daily, but follow your own prescription. Swallow whole with a glass of water; do not chew or crush. Take consistently with or without food. If a dose is missed, do not take an extra dose: take the next prescribed dose at the usual time.

Sources: nccp · smpc

Medicines, vaccines and food

Avoid grapefruit and St John’s wort. Ask the pharmacist to check all prescribed, over-the-counter and herbal products. Live vaccines should be avoided during treatment; agree vaccination with your team.

Sources: nccp · smpc

Procedures and radiotherapy

Discuss planned surgery, wounds and current or recent radiotherapy. Everolimus may affect healing and can worsen or reactivate radiation-related inflammation. The teams agree any interruption.

Sources: nccp · smpc

Tests and review

Baseline blood count, kidneys/liver, BUN, urine protein, lipids, fasting glucose and hepatitis B screening. Blood count, kidney and liver tests every four weeks. Lipids regularly and glucose as clinically indicated; scans and NET markers follow the consultant plan.

Sources: nccp · smpc

Mouth care

Use gentle mouth care and any prescribed alcohol-free mouthwash. Ask before buying a rinse; avoid products containing alcohol, peroxide, iodine or thyme. Report ulcers early.

Sources: nccp · smpc

Sickness and bowel support

Everolimus has minimal-to-low emetogenic potential. Routine premedication is not required by this regimen; agree any rescue anti-sickness or diarrhoea medicine with the team.

Sources: nccp · smpc

Infection prevention

Existing infection must be assessed and resolved before starting. Hepatitis B testing informs any specialist prevention plan. Vaccination and any antimicrobial prevention, including PJP prevention with certain steroid plans, are individual decisions.

Sources: nccp · smpc

Eating and energy

Report weight loss, poor intake or persistent fatigue. Ask for nutritional and symptom support; do not continue through severe symptoms simply to finish a pack.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

When to contact your team

Do not wait for the next scheduled blood test.

Breathing, infection and mouth symptoms

Contact oncology promptly for new or worsening cough, breathlessness or fever, mouth ulcers limiting eating or drinking, marked thirst, much less urine, severe diarrhoea or signs of infection. Severe breathing difficulty or collapse needs emergency help.

Sources: nccp · smpc

Dose holds and restarting

The team may hold or reduce treatment for side effects or abnormal tests. Record the reason and the new plan. Do not restart a held dose or increase a reduced dose yourself.

Sources: nccp · smpc

Safe storage and supply

Keep tablets in their original labelled packaging, out of children’s reach, and follow the pack storage instructions. Return unused tablets to the pharmacy as advised. Plan repeat prescriptions before your supply runs out.

Sources: nccp · smpc

Pregnancy, feeding and fertility

Tell the team before starting if pregnant, planning pregnancy or breastfeeding. Ask for the medicine-specific contraception duration and fertility advice, and record it in your plan.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

Understanding your particular NET

Several different features help the team choose treatment.

Where it started and where it is now

A neuroendocrine tumour (NET) starts in cells that receive signals and can release hormones. The original organ is the primary site. A pancreatic NET is different from the more common pancreatic adenocarcinoma. Stage describes how far the cancer has spread; the stage system depends on its original site. A tumour in the liver may be a NET that spread there from elsewhere.

Sources: nccn-patient

Differentiation, grade and Ki-67

Differentiation describes how the cells look under the microscope. Grade describes their growth activity. Ki-67 is a laboratory measure of the proportion of tumour cells showing a marker of cell growth and helps assess grade in relevant NETs. It is not the percentage of your body affected and is not a survival percentage. Ask for the exact pathology wording, grade and Ki-67 result, including the sample date.

Sources: nccn-patient

Grade is different from stage

A low-grade NET can have spread, and a high-grade tumour is not automatically stage 4. Well-differentiated grade 3 NET and poorly differentiated neuroendocrine carcinoma are different diagnoses with different treatment choices. These medicine guides must not be applied to another diagnosis simply because its name contains neuroendocrine.

Sources: nccn-patient

Functioning and nonfunctioning

A functioning NET causes a hormone-related syndrome. A nonfunctioning NET does not cause a recognisable excess-hormone syndrome, but it can still grow or cause symptoms by its size or location. Ask which description applies to you. A positive somatostatin receptor scan does not by itself mean the tumour is functioning.

Sources: nccn-patient · nci-pnet

Your diagnosis record

Keep the primary site, differentiation, grade, Ki-67 if reported, stage or sites of spread, hormone syndrome and receptor-scan result together in your passport. Ask the team to explain changes rather than trying to interpret one result in isolation.

Sources: nccn-patient

TREATMENT REFERENCE · 05

Making sense of tests and results

Safety tests, tumour tests and hormone tests answer different questions.

Blood tests for treatment safety

Blood counts measure infection-fighting cells, oxygen-carrying cells and platelets. Kidney and liver tests help the team assess how your body can handle treatment. The schedule for your medicine is shown separately. Attend these checks even when you feel well; a scan does not replace them.

Sources: nccn-patient · nccp

CT, MRI and receptor imaging

CT and MRI help show tumour size and where it is. A somatostatin receptor (SSTR) scan looks for receptor targets that certain treatments can attach to. An FDG-PET scan answers a different question about tumour activity and may be useful in selected cases. You will not necessarily need every scan. The specialist decides which test can change your care.

Sources: nccn-patient

Hormones and tumour markers are individual

The team may measure selected blood or urine markers when they answer a question about your NET or symptoms. These can include serotonin-related testing such as 5-HIAA for selected patients, or specific pancreatic hormones. Chromogranin A is not a universal test before every treatment. A changing marker alone does not prove cancer growth. Record why each test is being done, how to prepare and who will explain it.

Sources: nccn-patient · nci-net

Preparing for a test

Ask whether you need fasting, a timed urine collection, dietary preparation or a medicine review. Follow the laboratory's instructions for your particular test. Do not stop an acid-suppressing medicine, diabetes medicine or hormone-control treatment yourself. Tell the team about kidney problems, allergies and all medicines before a scan or blood test.

Sources: nccn-patient · nci-net

Heart and inherited-risk checks

A heart ultrasound may be needed for particular treatment risks or to assess heart-valve problems associated with a hormone syndrome. It is not required on the same timetable for every NET. Depending on the diagnosis, age and family history, the team may offer genetic counselling; some NETs occur with inherited conditions such as MEN1. A tumour gene test and an inherited-risk test are not interchangeable.

Sources: nccn-patient

Who does what, and when?

Use the passport to write each test, its purpose, planned date, booking service, result-review contact and next step. Your consultant sets response-scan and marker intervals. The postoperative schedules in general guides are not automatically the right schedule while you are taking these medicines.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 06

Hormone symptoms and day-to-day changes

A new symptom may come from the NET, treatment or another illness.

Build a picture of what is normal for you

Record your usual bowel pattern, flushing, pain, appetite, weight and energy before starting. Note what changes, how long it lasts, what you were taking and how it affects daily life. This helps the team distinguish a hormone problem from treatment toxicity without assuming that every symptom is cancer progression.

Sources: nccn-patient

Hormone-related symptoms

Depending on the tumour, excess hormones may cause flushing, frequent watery stools, wheezing, acid-related pain or blood-sugar disturbances. An insulin-producing tumour can cause low blood sugar. Not every NET causes these problems. Ask which symptoms matter for your diagnosis and whether you need a separate rescue plan.

Sources: nccn-patient

Hormone treatment is a separate part of the plan

Somatostatin analogue injections and other symptom treatments may continue alongside anticancer treatment when prescribed. They may have their own appointment dates. Never stop them simply because tablets are paused. PRRT requires a carefully coordinated interruption and restart plan, which the nuclear-medicine service must supply.

Sources: nccn-patient · nccp

When a change is urgent

Follow your oncology emergency plan for sudden or severe symptoms. Collapse, severe breathing difficulty, chest pain, confusion or uncontrolled bleeding needs emergency assessment. Severe diarrhoea, persistent vomiting, dehydration, fever or sudden illness needs urgent oncology advice. Do not wait for a scan, an online answer or the next clinic visit.

Sources: nccp

Procedures and hormone syndromes

Tell the surgical, dental, anaesthetic or imaging team if you have a hormone-producing NET or a previous hormone crisis. Ask your specialist whether special preparation is needed. Carry your current medicines and emergency plan; the wallet card supports this information but does not replace specialist instructions.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 07

Your overall plan and choices

Your priorities belong in the treatment plan.

What are we trying to achieve?

Ask whether the main aim is to slow growth, reduce tumour size, relieve hormone symptoms, make another treatment possible or protect day-to-day quality of life. The aim and likely benefit depend on your particular NET and previous treatment. A stable scan may represent useful control even without tumour shrinkage.

Sources: nccn-patient

How treatments fit together

Some people need surgery, hormone-control treatment, tablets, chemotherapy, PRRT, liver-directed treatment or external radiotherapy at different points. These are not all automatically part of your regimen. Ask the multidisciplinary team to draw your own sequence and name the clinician coordinating it.

Sources: nccn-patient

Choosing between options

Ask why this option suits your tumour site, grade, growth rate, symptoms, receptor status and other health conditions. Discuss alternatives, the option of observation when appropriate, supportive care, and any suitable clinical trial. A treatment listed in a US guide is not a guarantee of availability or funding in Ireland.

Sources: nccn-patient

Evidence is about groups

A trial can measure time before cancer worsens, tumour shrinkage or survival. These are different outcomes. A group result does not predict your own response or lifespan. Ask how closely the study patients match your diagnosis and what benefit would make the treatment worthwhile to you.

Sources: nccn-patient

Review points and shared decisions

Agree when you will review symptoms, tests and scans, and what would lead to continuing, delaying, changing or stopping treatment. Discuss work, caring duties, travel and the effects you most want to avoid. You can revisit your choices as your situation changes.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Eating, energy, practical help and support

Supportive care can be used alongside treatment from the start.

Food, weight and digestion

Tell the team about weight loss, poor appetite, early fullness, persistent diarrhoea or greasy stools. Ask for a dietitian familiar with NETs and your surgery or medicines. Different causes need different support; do not assume every bowel symptom needs the same diet or medicine. Ask whether digestive-enzyme assessment or replacement is relevant to you.

Sources: nccn-patient · nci-pnet

Fluids and safe activity

Follow your individual fluid advice, especially with diarrhoea, kidney disease or heart problems. Balance manageable activity with rest. Tell the team if fatigue, dizziness, pain or breathlessness limits usual tasks; these symptoms can have treatable causes.

Sources: nccn-patient · nccp

Emotions, relationships and quality of life

Living with repeated tests and treatment can affect sleep, mood, relationships and confidence. Ask about psychological support, sexual health, fertility advice, symptom-control services and support for carers. Supportive or palliative care aims to improve comfort and daily life and can accompany active anticancer treatment.

Sources: nccn-patient

Practical help

Ask your nurse or social worker about transport, work, finances, home support, caring responsibilities and local NET support organisations. A general resource list may include services in other countries; your team can identify options available to you.

Sources: nccn-patient

A record that works without a phone

Keep the printed schedule, medicine list, tests, contact numbers and written emergency instructions together. The full handbook contains the information in sequence for offline reading. The QR code is an optional route to more information; it is not needed to call for help or follow your paper plan.

Sources: nccn-patient

Questions to take to the next visit

What is my exact diagnosis and grade? What is treatment trying to achieve? Which tests are mandatory for this medicine, and which depend on my NET? Who reviews results? What symptoms mean call now? What support would help me live as well as possible?

Sources: nccn-patient

Evidence depends on the NET site and setting

The principal trials assessed time before disease worsened.

Everolimus delayed progression compared with placebo in these trial populations. That does not mean cure or predict an individual response.

Sources: radiant3 · radiant3-os · radiant4

PREPARE FOR THE CONVERSATION

Questions worth asking

Which NET indication and treatment aim apply to me?

Ask your team to name the exact pancreatic or nonfunctional GI NET indication, the treatment aim and how it fits with your previous care. This guide does not cover the separate kidney-cancer indication.

When are my four-week blood tests and glucose/lipid checks?

NCCP specifies blood count, kidney and liver tests every four weeks. Lipids are checked regularly and fasting glucose as clinically indicated. Write your actual test dates and result-review contact in the passport.

What mouthwash or rescue medicines have you prescribed?

Ask the team to write the mouth-care and any rescue medicine prescription. Alcohol-free mouthwash may be required; there is no automatic routine anti-sickness premedication in this regimen.

What should I do about a new cough, fever or breathlessness?

Contact oncology promptly. Infection and non-infectious lung inflammation can look similar and need assessment. Severe breathing difficulty or collapse needs emergency help.

How does this fit with my hormone-control treatment?

Ask the NET team to list any separate hormone-control medicine and its schedule. Do not stop injections or other prescribed treatment simply because everolimus is held.

When will we review scans, symptoms and my priorities?

The consultant sets the imaging and marker plan and reviews benefit, side effects and your goals. Ask for the next decision date and who will explain the results.

Try explaining the plan back: Can you explain the daily tablet plan, the four-week blood checks, mouth care and which breathing or infection symptoms need prompt contact?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00320 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00320 v6 · current Irish regimen
- EMA product information and patient leaflet
- NCCN Guidelines for Patients: Neuroendocrine Tumors, 2026 · education reference
- NCI PDQ: gastrointestinal NETs · selected marker background
- NCI PDQ: pancreatic NETs · hormone and digestive background
- RADIANT-3 primary pancreatic NET report
- RADIANT-3 final overall-survival analysis
- RADIANT-4 nonfunctional GI/lung NET trial

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6718/320_Everolimus.pdf

https://www.ema.europa.eu/en/documents/product-information/afinitor-epar-product-information_en.pdf

https://www.nccn.org/patients/guidelines/content/PDF/neuroendocrine-patient.pdf

https://www.cancer.gov/types/gi-neuroendocrine-tumors/hp/gi-neuroendocrine-treatment-pdq

https://www.cancer.gov/types/pancreatic/hp/pnet-treatment-pdq

https://pubmed.ncbi.nlm.nih.gov/21306238/

https://pubmed.ncbi.nlm.nih.gov/27621394/

https://pubmed.ncbi.nlm.nih.gov/26703889/